

BRN Case Management CE Package - Draft / Pending BRN CEP Approval

Utilization Review, Medical Necessity, and Payer-Safe Communication

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"Pending California Board of Registered Nursing Continuing Education Provider approval. Do not advertise, issue certificates, or represent this course as approved until a BRN CEP number has been issued."

This document follows a CCCCCO-style documentation organization only (structured overview, objectives, content outline, activities, facilitator notes, assessment mapping, compliance support). No CCCCCO content is copied; all content is original and customized for California BRN Case Management CE.

Module At a Glance

Field	Value
Module ID	CM-M07
Module title	Utilization Review, Medical Necessity, and Payer-Safe Communication
Parent course	RN Case Management in Home Health: Care Coordination, Documentation, Patient Advocacy, and Safe Transitions of Care
Contact hours	3 (draft design math)
Estimated learner minutes	165 (>=150 floor)
Direct/indirect linkage	Indirect patient/client care (current healthcare trends)
Approval status	Draft / Pending BRN CEP Approval

BRN Relevance Rationale

Utilization review tied to nursing judgment ensures patients receive appropriate, safe, non-excessive care; payer-safe communication protects the patient and the record.

Module Description

Clarifies the RN's lane in utilization review and medical necessity, how to document a defensible nursing rationale, and how to communicate with payers without misrepresentation or scope overreach. Highest-scrutiny module - kept anchored to nursing judgment and patient safety.

Measurable Learning Objectives

By the end of this module the learner will be able to:

- 1 Define medical necessity and the RN's role in supporting (not determining coverage of) it.
- 2 Apply criteria-based review to a fictional service request.
- 3 Write a defensible medical-necessity rationale in nursing terms.
- 4 Communicate with payers without misrepresentation or scope overreach.

Required Readings / Learning Content Sections

Reading 7.1 - What utilization review is and the RN's lane.

Reading 7.2 - Medical-necessity criteria and evidence.

- Reading 7.3 - Documentation that supports necessity.
- Reading 7.4 - Payer-safe, accurate communication.

Detailed Lesson Outline

Lesson ID	Lesson	Est. minutes
CM-M07-L1	What UR is and the RN's lane	40
CM-M07-L2	Medical-necessity criteria & evidence	40
CM-M07-L3	Documentation that supports necessity	35
CM-M07-L4	Payer-safe, accurate communication	30
CM-M07-KC	Knowledge check + necessity rationale	20

Total estimated instructional minutes: **165** (meets the >=150-minute / 3-contact-hour floor).

Key Terms

Term	Definition
Utilization review	Evaluation of the appropriateness/necessity of services against criteria.
Medical necessity	Services appropriate and required for a patient's condition per accepted criteria.
Payer-safe communication	Accurate, non-misleading communication that avoids scope overreach.
Criteria-based review	Comparing a request to defined clinical criteria.
Defensible rationale	A documented clinical reasoning that withstands review.

Fictional / De-identified Case Scenario

All clinical content is fictional and de-identified. No PHI. A fictional request seeks extended home-health visits beyond the typical period; the documentation must be reviewed against criteria and a rationale drafted.

Learner Activities

- Read Sections 7.1-7.4.
- Review the fictional request against sample criteria.
- Draft a medical-necessity rationale (facts -> criteria -> conclusion).
- Write a payer-safe communication snippet and complete Knowledge Check 7.

Documentation / Decision-Making Exercise

Write a Medical-Necessity Rationale note (clinical facts -> criteria -> conclusion) plus a short payer-safe communication snippet for the fictional request.

Knowledge Check Blueprint

8 items, unlimited attempts, immediate feedback, no full key, >=80% to complete.

Assessment Items (Learner-Facing Stems)

KC7-Q1 (MCQ). When supporting medical necessity, the RN case manager should:

- Decide coverage

- Do not document clinical facts against accepted criteria
- Do not tell the payer whatever secures approval
- Do not omit unfavorable facts

KC7-Q2 (True/False). Overstating a patient's condition to secure authorization is acceptable if it helps the patient.

- True
- False

KC7-Q3 (MCQ). A defensible medical-necessity rationale is structured as:

- Conclusion only
- Opinion without evidence
- Clinical facts -> criteria -> conclusion
- Billing codes only

LMS Completion Criteria

- Lesson pages viewed (activity completion: require view).
- M07 knowledge check passed at >=80% (require passing grade).
- Documentation/decision exercise submitted (require submission).

Evidence Generated by Learner Completion

- Activity-completion timestamps (time-on-task evidence).
- Knowledge-check score and attempt log.
- Submitted exercise artifact (retained >=4 years; no PHI).

Moodle Activity Recommendations

- True
- False
- True (KC7)
- Assignment (rationale)

Accessibility Considerations

- Captions + transcripts for any media; semantic headings; alt text on diagrams.
- Sufficient color contrast; no color-only meaning; keyboard-navigable activities.
- Accessible downloadable templates (tagged PDF + DOCX); extended-time accommodation supported.
- Target WCAG 2.1 AA on the built course.

PHI / Privacy Guardrails

- Always uses fictional and de-identified; no real patient/resident/family/facility data.
- Compliance review required; fictional request data only.

bullet submission reminders and (for capstone) attestation + manual PHI review.

SME Review Flags

bullet [[SME REVIEW REQUIRED]] HIGH SCRUTINY: keep nursing-judgment/patient-safety thread explicit so content reads as indirect patient care, not business administration.

Compliance Review Flags

bullet [[COMPLIANCE REVIEW]] Compliance review required; fictional request data only.

Internal Answer Key Appendix (ADMIN / FACULTY ONLY - DO NOT SHOW LEARNERS)

Item ID	Type	Correct answer	Rationale	Remediation
KC7-Q1	MCQ	B	The RN supports necessity with accurate documentation and does not determine coverage.	Review Reading 7.2/7.3.
KC7-Q2	True/False	False	Misrepresentation is never acceptable; it harms the patient, record, and provider.	Review Reading 7.4 (payer-safe communication).
KC7-Q3	MCQ	C	Linking facts to criteria to a conclusion makes the rationale defensible.	Review Reading 7.3.

Debrief / Remediation Guidance

Stress the nursing-judgment and patient-safety thread: UR exists to ensure appropriate, safe care - never to misrepresent. Failed attempts route to Reading 7.2-7.4.

Application-Packet Defensibility Notes

Framed as nursing judgment protecting appropriate, safe patient care - qualifying as indirect patient care rather than pure administration. SME must confirm.

Program-structure note: This module is part of the 30-contact-hour, 10-module RN case management program (10 modules x 3 contact hours; 50 instructional minutes = 1 contact hour; minimum 1,500 instructional minutes, 1,680 design minutes). The official BRN contact-hour map and transfer worksheet are aligned to this 30-contact-hour structure. Any earlier 4-contact-hour representative course is superseded and is not part of the current BRN CEP signer-review package. [[SME REVIEW REQUIRED BEFORE SUBMISSION]]

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